



KEMENTERIAN KESIHATAN MALAYSIA

TAJUK: CHRONIC DIALYSIS TREATMENT POLICY

NOMBOR DOKUMEN: HS/P&P/13C (CHRONIC DIALYSIS TREATMENT POLICY)

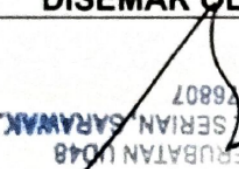
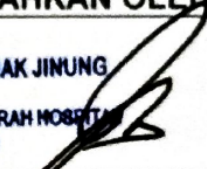
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Tarikh Pindaan	No Pindaan	Penerangan Pindaan	Tandatangan anggota yang membuat pindaan polisi	Tandatangan pegawai yang meluluskan pindaan polisi
30.10.2023	9.3.9	Penambahan kepada Pengurusan Pesakit COVID-19	MAT BIN POZAN No. Daftar 5910 Penolong Pegawai Perubatan Unit Haemodialysis	DR. TEE HEN PAKAR PERUBATAN HOSPITAL SERIAN SARAWAK MMC NO. 76807

JUMLAH SALINAN	SENARAI PENERIMA
1	Pengarah Hospital Serian
2	Unit Heamodialisis Serian
3	Unit Kualiti Serian

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1.0 INTRODUCTION

Nephrology is one of the major clinical specialty services in the Department of Medicine at Hospital Serian. RRT for ESRF is an integrated program consisting of Haemodialysis, Peritoneal Dialysis, Renal Transplantation and also Palliative Nephrology. Patients can and shall be transferred from one RRT treatment modality to another as and when necessary. This will ensure the care of ESRF patients shall be a seamless and continuous. All ESRF patients shall be co-managed with the Nephrology Team in Sarawak General Hospital . All MOH Hospitals shall establish a Haemodialysis unit. The unit shall provide outpatient and inpatient Haemodialysis treatment.

*Subject to condition and stability of individual patient

2.0 VISION

Our Unit will never GIVE UP in providing quality health care services and aim to be a PLACE where all kidney failure patients will want to be.

3.0 MISION

To Provide comprehensive and high quality Haemodialysis services through trained staff.

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4.0 OBJECTIVES

4.1 GENERAL OBJECTIVES

To ensure effective and efficient management of patients with chronic dialysis treatment.

4.2 SPECIFIC OBJECTIVES

4.2.1 To Provide Outpatient and Inpatient (where suitable) Haemodialysis Services.

4.2.2 To Provide Education and Counselling to Haemodialysis patients with regards to their management and medications.

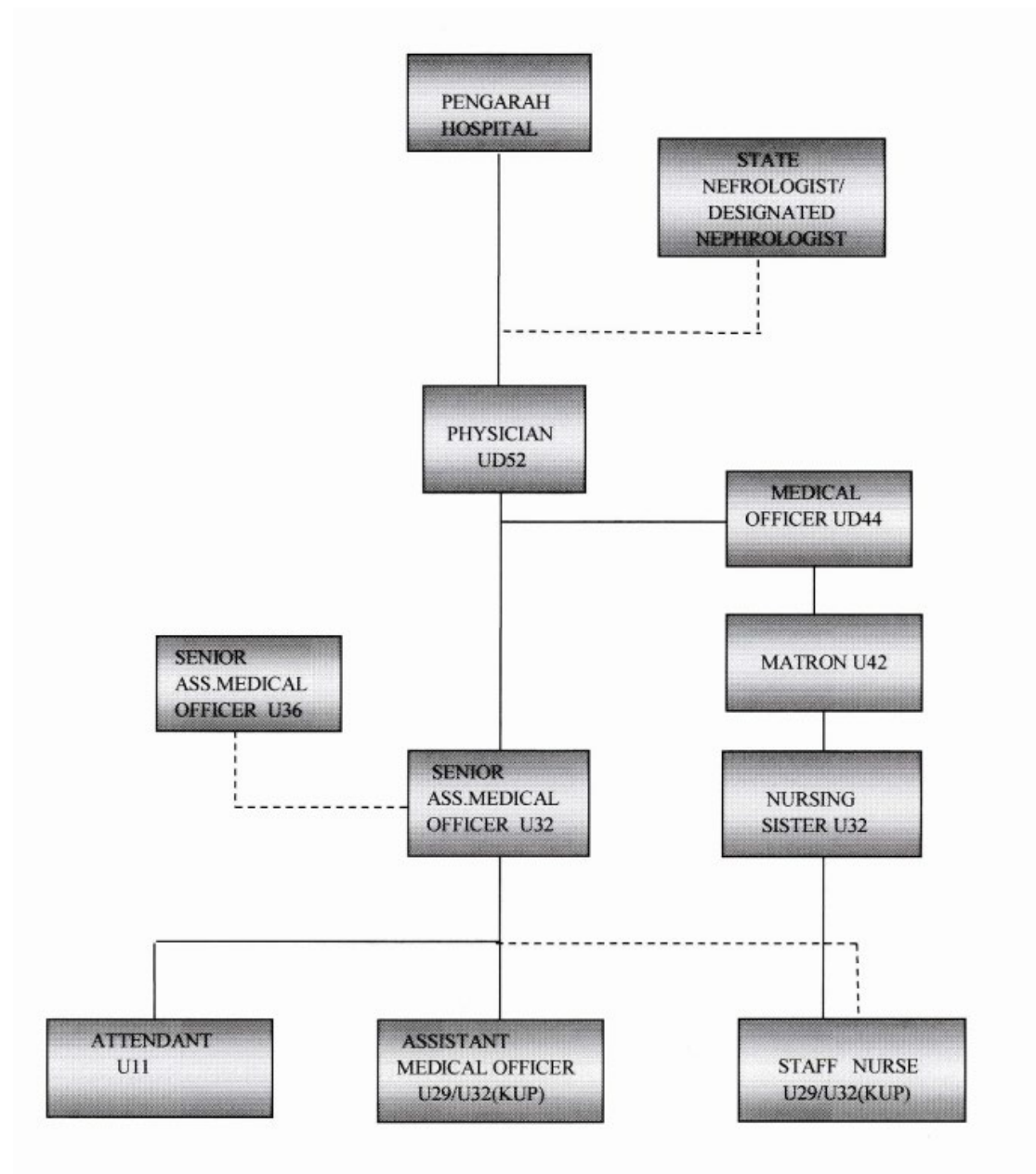
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5.0 HAEMODIALYSIS UNIT ORGANISATION CHART

HAEMODIALYSIS ORGANISATION CHART AT SERIAN HOSPITAL



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6.0 GOVERNANCE

6.1 Governance of the Haemodialysis Centre

The governance of the Haemodialysis Centre shall be the responsibility of the Physician In Charge.

6.2 Roles and Responsibilities

The Roles and Responsibilities of the Physician:

- i. Ensure Haemodialysis patients are accessible to appropriate medical care, as and when needed;
- ii. Ensure that the Centre has a nephrologist to assume all or part of the medical care of the patients;
- iii. Involvement in development and periodic review of policies and procedures;
- iv. Performance assessment and improvement programme;
- v. Staff education and performance;
- vi. Ensure patient education programme is available;
- vii. Ensure proper functioning and maintenance of all facilities and equipment.

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7.0 ACCESS TO CARE

Patients with end-stage renal disease shall have access to safe, efficient and effective haemodialysis treatment.

7.1 Acceptance of Patients into Haemodialysis Centre / Patient Selection Policy

7.1.1 Selection of patients for long term haemodialysis in the haemodialysis unit shall be under the purview of a selection committee.

7.1.2 This committee shall consist of specialists in the Department of Nephrology and senior allied health staff from the dialysis unit. A consultant nephrologist shall chair this selection committee.

Refer to **Appendix 1 - Policy on Selection and Acceptance of Patients**

7.2 Haemodialysis Clinic

7.2.1 The HD clinic is dedicated for the follow up of patients that are on chronic haemodialysis in the Ministry of Health.

7.2.2 The objective of the clinic is to review HD patients every 3 months or more frequently if indicated to ensure that the patients receive optimal care

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7.2.3 During each clinic visit

- i. The patients shall receive a physical examination including an examination of the vascular access.
- ii. The patient's dialysis charts and laboratory tests shall be reviewed including an assessment of dialysis adequacy.
- iii. Complications of ESRF and haemodialysis such as cardiovascular disease, renal bone disease, anaemia and malnutrition shall be assessed.
- iv. The patients shall received dietary advice and be referred to dietician when necessary.

7.3 Access to Other Medical Care

7.3.1 The Haemodialysis Centre is a part of Hospital Serian and hence Haemodialysis patients shall have access to Hospital Serian should they require any other medical treatment

7.3.2 The patients shall be referred to a Medical Officer in charge should they require any other medical care.

This would also include Dietitian referrals, Physiotherapy and any other relevant treatment needed

A record of the patient referrals to interdepartments shall be kept in the patients Folder

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7.4 Access to the National Deceased Donor Kidney Transplant waiting list

7.4.1 Refer to Appendix 2 - *Policy on Renal Transplantation*

7.4.2 The centre under the advise of a nephrologist shall provide information about kidney transplantation (including deceased and live donor kidney transplant)

7.4.3 Living Related Donors

The types of living renal donors are:

i. Live related donor

Live related donor implies a genetically related first and second degree family member. Other members shall be assessed by the Unrelated Transplant Approval Committee (UTAC) in the Ministry of Health.

II. Live emotionally related donor

The live emotionally related donor shall be the legally registered spouse of the recipient

iii. Live unrelated donor

Live unrelated renal donation shall not be accepted. However in exceptional cases, the case shall be referred to UTAC for approval before proceeding with the work-up.

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7.4.4 Deceased Donor Renal Transplant waiting list

All suitable potential recipients shall be placed on the national deceased donor waiting list. Patients are selected based on the criteria established by the Malaysian organ Sharing System.

8.0 HUMAN RESOURCE DEVELOPMENT AND MANAGEMENT

8.1 The Haemodialysis Team

The Centre shall have adequate number of qualified and trained staff as well as other supporting staff commensurate with the workload The Haemodialysis Team shall consists of:

1. Nephrologist
2. Physician
3. Medical Officer
4. Senior AMO
5. Trained Allied Health Staff
6. Attendants
7. Other relevant departments including services of medical social worker Dietician, renal pharmacist as required.

8.2 HDU Manager

- 8.2.1 The haemodialysis unit shall be headed by a Senior Assistant Medical Officer as the team leader.
- 8.2.2 It is preferred that the person had a undergone a post basic renal course in order to provide effective guidance to the staff under his supervision.

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8.2.3 Administration of dialysis related activities e.g. quality control, ordering stocks, stock flow tracking and trending, budgeting etc and setting up of the unit shall be under the HD manager

8.3 Assistant Medical Officer and Staff Nurse

8.3.1 The staff to patient ratio is 1:4

8.3.2 Individuals with suitable qualifications and had preferably completed a six month post basic renal nursing course. His/Her duties are:

- i. To provide haemodialysis treatment to patients with acute and end stage renal disease.
- ii. To perform on-call duties as per roster
- iii. To recognize acute complications of haemodialysis, take remedial measures and inform doctor.
- iv. To counsel and educate patients on dietary control, medication compliance and basic functions of haemodialysis machine.
- v. To adhere to infection control policies.
- vi. To comply with nephrology practice protocols and guidelines.
- vii. To trace laboratory investigations , compile the haemodialysis records and document the patients' progress in the patients' case note.
- viii. To collect patients' data for National Renal Registry and maintain and update potential cadaveric renal transplant list
- ix. To attend CME activities.
- x. To train patients in self care
- xi. To assist in skills assessment

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8.4 Other support staff

The Centre may employ other non-clinical staff whose roles and responsibilities are clearly defined. Other support staff are appointed and clearly assigned to support the service needs

9.0 POLICIES AND PROCEDURES

9.1 CARE OF THE HAEMODIALYSIS PATIENT

9.1.1 Clinical Care

9.1.1.1 Documentation on Nephrologist/HDU Clinic review, medications and other appropriate treatment to be kept in Haemodialysis Unit.

9.1.2 Haemodialysis Prescription

9.1.2.1 All patients should have a prescription for haemodialysis treatment shall be reviewed at least three monthly.

9.1.3 Haemodialysis Outcome

9.1.3.1 All patients shall have haemodialysis outcome indices monitored at least three monthly

9.1.3.2 Investigations to be done include:

- i. Studies on anaemia
- ii. Nutritional status
- iii. Adequacy of dialysis
- iv. Mineral Metabolism
- v. Virology- studies

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9.2 ETHICAL PRACTISE AND PATIENT & FAMILY RIGHTS

The Centre shall establish ethical guidelines that promote appropriate, safe and efficacious haemodialysis treatment. It shall have policies supporting patients' rights as well as informing them of their responsibilities.

9.2.1 Confidentiality of Patient's Personal and Medical History

9.2.1.1 Information on the patient's personal and medical history shall be kept confidential at all times.

9.2.2 Informed Consent

9.2.2.1 There shall be informed consent before patient is started on haemodialysis Treatment.

9.2.2.2 The informed consent document shall include nature of treatment, short and long term potential complications, cost of care and access to other non-dialysis care.

9.2.2.3 Hospital Serian Consent Form before initiating Haemodialysis attached in Appendix 4 - *Hospital Serian Consent Form for Initiating Dialysis Treatment for New ESRD Patient*.

9.2.3 Patients Rights and Responsibilities

9.2.3.1 The Centre shall have a guide on the rights and responsibilities of the patient undergoing dialysis to ensure his/her well-being and a best possible outcome. This shall be communicated to the patient when he/she starts treatment at the Centre.

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9.2.4 Adequate Monitoring

9.2.4.1 The Physician shall ensure adequate monitoring of patients during dialysis and subsequent patient care.

9.2.5 Centre Closure

9.2.5.1 In the case of closure of the Centre, the Physician together with the Nephrologist shall ensure there is continuity of care of all patients including transfer of patients to another Haemodialysis Centre.

9.3 PREVENTION AND CONTROL OF INFECTION

9.3.1 Infection Control System and Processes

9.3.1.1 The Haemodialysis Unit shall follow the policies and practices as determined by the National Advisor of Nephrology services.

9.3.1.2 An infection control team shall be identified to activate, regulate, monitor and report infection control activities including staff training, case detection, documentation and audit activities.

9.3.1.3 Strict adherence to the guidelines for universal precautions by the staff shall be practiced at all times.

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9.3.1.4 Staff Screening

Staff with Hepatitis B, C positive are not allowed to work in the unit.

Staff to change into blue attire and slippers to prevent uniform and shoes from contamination.

All HDU staff MUST have Hepatitis B screening, if no antibody the staff shall be vaccinated according to schedule.

All new staff MUST be trained on Universal Precaution and procedure once they are in the unit and their biohazard are negative.

Standard precaution should be maintained throughout the procedure or treatment from patient to patient.

9.3.1.5 Refer to Appendix 5 for *Infection Control Policy In the Haemodialysis Unit*

9.3.1.6 Refer to Appendix 6 for *Needle Stick Injury Protocol*

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9.3.2 Pre-admission Screening

Patient Screening

All new patient subjected to chronic haemodialysis treatment must have their blood test done. Eg: FBC VDRL, HIV , screening for Hepatitis B, C and antibody before starting HD treatment in the unit. Patient must be vaccinated if their antibody is negative.

9.3.2.1 For Screening of Blood Borne Viral Infection ,Refer to Appendix 7- Policy on *Screening and Monitoring Blood Borne Viral Infections*.

9.3.3 Monitoring of Infections

9.3.3.1 There shall be on going monitoring of infections as per policy in stated in Policy on Screening and Monitoring Blood Borne Viral Infections.

Other infection monitoring shall be done including CRBSI - Catheter related Blood Stream Infection.

Refer to Appendix 10 - *CRBSI Reporting And Surveillance Form*

One form shall be filled out for one patient each time a CRBSI event occurs. For one patient, if multiple CRBSI events occur and are separate events – then separate forms for each event are to be filled out.

For each CRBSI Reporting and Surveillance Form filled out, there will be another copy sent to infection control unit for recording.

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9.3.4 Policy on management of Infected Patients

9.3.4.1 For patients Hepatitis B and HIV, Refer to Appendix 8 - Policy on Handling of Infected Patients.

9.3.5 Screening for Visiting Patients and Centre's patients who have temporary dialysis elsewhere

9.3.5.1 Patients from other centres who request to dialyse at the Centre shall undergo screening for Blood Borne Viral Infections.

9.3.5.2 The Centre's own patients who return from dialysis treatment at other centres shall undergo similar screening.

9.3.5.3 Refer to Appendix 9 - *Policy and Procedures on Screening of the Centre's Patient who Temporary Dialyse Outside the Centre.*

9.3.6 Training of Staff in Infection Control

9.3.6.1 Designated staff with post basic renal nursing oversee the prevention and control measure in the Centre.

9.3.7 Documentation of Infections

9.3.7.1 Complete documentation of infection complications within the Centre" which include Catheter Related Blood Stream Infections (CRBSI) and Blood Borne Viral Infections.

9.3. 7.2 Refer to Appendix 10 - *CRBSI Reporting Form.*

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9.3.8 Management of Infected Patients

9.3.8.1 Refer to Appendix 8 - *Policy on Handling Infected Patients*

9.3.9 Management of COVID -19 Patient

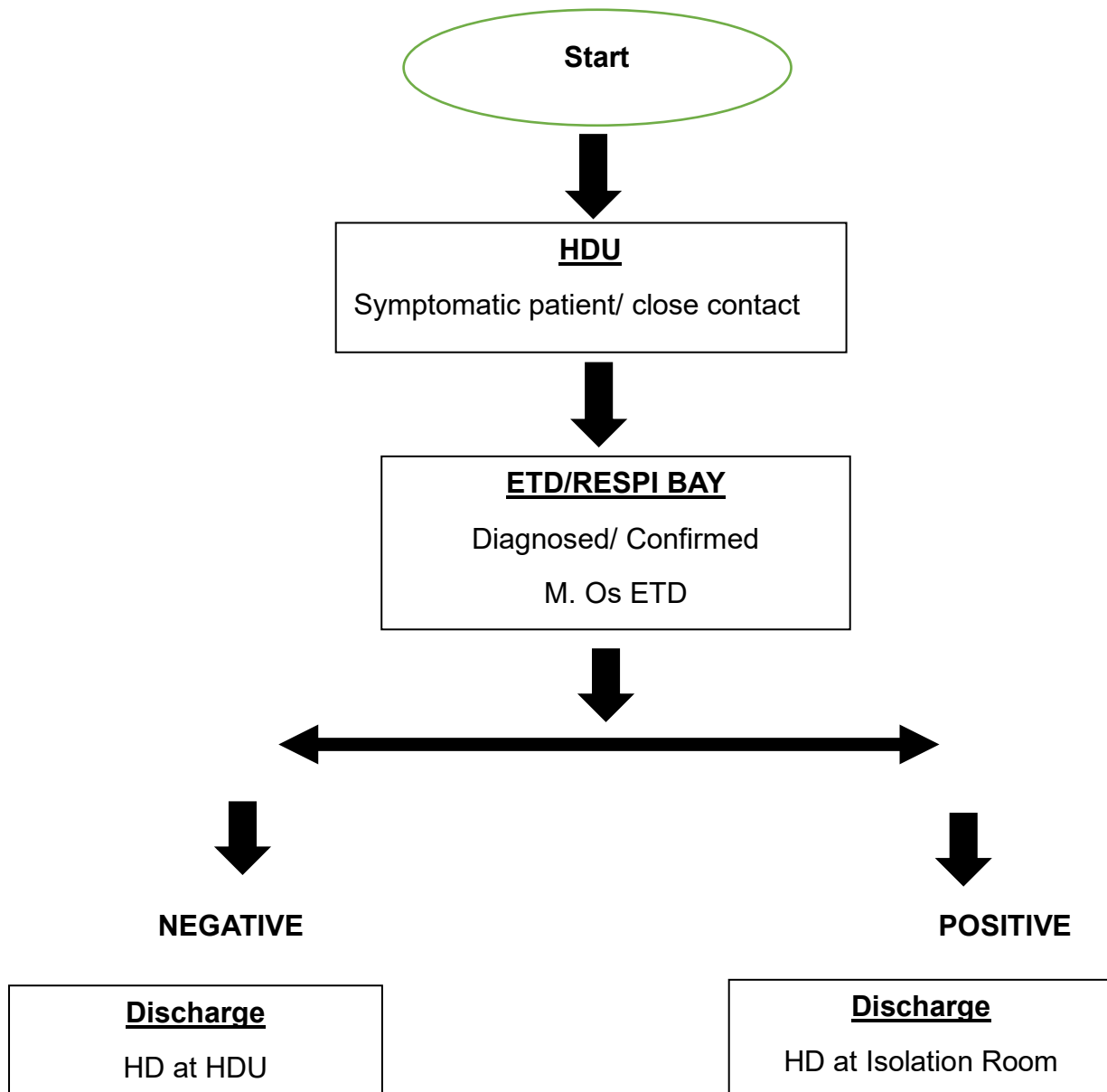
9.3.9.1 All patients with suspected COVID-19 infection symptoms prior to haemodialysis session will be advised for self-test and for further isolation and notification as per current COVID-19 protocol if COVID- 19 tested positive.

9.3.9.2 COVID-19 positive patient will be undergoing dialysis in Isolation Room, Hospital Serian throughout quarantine period.

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**WORKFLOW FOR SCREENING AND TREATMENT OF COVID-19 IN
HAEMODIALYSIS PATIENTS**



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10.0 FACILITIES AND EQUIPMENT

10.1 The Centre shall comply with the requirements of the local authority, National Standards for Haemodialysis, Medical Devices Act, and any other relevant regulatory requirements.

10.1.1 Structural Design and Location of Haemodialysis Centre

- 10.1.1.1 Floor plan should be available and displayed in the Centre.
- 10.1.1.2 The Haemodialysis unit is located within the hospital grounds and easily accessible to the patients who receive the treatment on a regular ambulatory basis.
- 10.1.1.3 It is also at a location with easy access to vehicles delivering machines consumables and disposables with availability of adequate water pressure for the HD, Reverse Osmosis (RO) reprocessing machine and security for patients, staff and equipment.

10.1.2 Equipment Standards and Maintenance

- 10.1.2.1 Water System
- 10.1.2.2 Adequate uninterrupted water supply is a prerequisite to effective haemodialysis treatment.
- 10.1.2.3 Every haemodialysis treatment shall use water treatment by RO system to ensure purity of the water, ultrapure water treatment system is preferred.

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10.1.2.4 The water quality shall meet the Association for the Advancement of Medical Instrumentation (AAMI) standards (AAMI RD62:2006). Appendix 11

10.1.2.5 Regular chemical analysis of water quality shall be carried out 3 to 6 monthly.

10.1.3 Equipment (Refer Appendix 12) and Consumables

10.1.3.1 Haemodialysis Machines

10.1.3.2 The MOH Nephrology service shall provide HD machines which are modern, cost effective and with all the monitoring and other features that will ensure efficient and safe haemodialysis.

10.1.3.3 Only machines approved by major regulatory authorities e.g FDA, EMEA and Japan Ministry of Health of shall be purchased.

10.1.3.4 All HD machines shall be serviced regularly according to the manufacturer's Recommendations.

10.1.3.5 Records of Filter change date shall be kept at each Haemodialysis machine.

10.1.3.6 A daily checklist of Haemodialysis machines is available in the Haemodialysis Unit.

10.1.4 Consumables

10.1.4.1 Synthetic membrane dialysers may be reprocessed using appropriate and safe reprocessing techniques. Dialysers with biocompatible synthetic membrane are preferred.

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10.1.4.2 Blood lines shall be procured that can be universally used in all types of machines. Bloodlines shall not be reused.

10.1.4.3 Dialysate shall be formulated and used according to current international recommendations. There shall be a number of different formulations to meet specific patient needs. Dialysate concentrate in the powder form is preferred.

10.1.5 Reprocessor Machine

10.1 .5.1 Automated reprocessors with pressure check and fibre bundle volume measurement shall be used.

10.1.5.2 A Record of Reprocessed Dialyzers details shall be kept inside the Reprocessing Room.

10.1.6 Power Supply

10.1.6.1 Continuous power supply for uninterrupted haemodialysis treatment.

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11.0 SAFETY AND PERFORMANCE IMPROVEMENT ACTIVITIES

11.1 The Centre has a framework of quality objectives and the processes to achieve these objectives

11.1.1 Plan for Performance Improvement Activities

- 11.1.1 .1 The Physician shall ensure staff involvement in the continuous safety and performance improvement activities.

There shall be Physician / HDU MO Rounds monthly - to review certain patients with relevant issues and any other HDU related issues. The rounds shall be set at every fourth Thursday of the month* (subject to change).

Records of Physician/Mo Rounds to be kept in Blue File in HDU. This file should not be removed from HDU.

11.1.2 Training in Performance Improvement Activities

- 11.1.2.1 Staff are trained in performance improvement activities and undergo continuous education.

There shall be regular Staff and Patient Education Activities planned and Performed.

Time table for CME to be kept in the respective folders in HDU.

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11.1.3 Documentation of Performance Improvement Activities

11.1.3.1 Documentation of Quality parameters

Patients should have their minimum laboratory lab tests done according to Policy on Minimum Laboratory Tests done for Haemodialysis Patients. Refer Appendix16

11.1.3.2 Refer to Appendix 13 - *Quality Improvement Activities*.

11.1.3.3 Records of Incidents Reporting and Mandatory incident Reporting to MOH:

- i. Viral Hepatitis and HIV Seroconversion
- ii. Intradialytic death in chronic stable dialysis patient.

11.1.3.4 Certificate of compliance from National Renal Registry.

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APPENDICES

APPENDIX 1

POLICY ON SELECTION AND ACCEPTANCE OF PATIENTS

1. Selection of patients for long term haemodialysis in the haemodialysis unit shall be under the purview of a selection committee.
2. This committee shall consist of specialists in the Department of Nephrology and senior allied health staff from the dialysis unit. A consultant nephrologist shall chair this selection committee. The selection criteria would depend on:
 - i. Waiting time
The duration on the waiting list is the major criteria for acceptance.
 - ii. Priority group
The following groups of patients will be given top priority and will be accepted into the MOH dialysis programme once vacancy is available:
 - a. Patients who had undergone a living related or local cadaveric renal transplantation at MOH hospitals and now require dialysis because of failed graft.
 - b. Dialysis patients in MOH hospitals who require a change in dialysis modality.
 - c. Patients transferred from other MOH dialysis units.
 - d. Children

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- iii. Government employment Vacant dialysis slots will be distributed among government (and their dependents) and non government employees/others.
 - iv. Co-morbid factors and potential for rehabilitation
 - v. Distance to the haemodialysis unit
 - v. Social economic status.
 - vii. Number of dependants (unemployed spouse and children below 18 or children who are studying)
 - viii. Age of patient.
 - ix. Availablirty of dialysis opportuniQr, resources or assistance (Baitulmal, SOSCO, employer, JPA etc).
 - x. Dialysing at subsidised/ NGO units.
 - xii. Dependency.
3. The above selection criteria cannot be enforced strictly as selection of patients for RRT involves multiple clinical and non-clinical factors including patient's choice.
 4. Hence these criteria remain a guide and other criteria may be considered at the discretion of the selection committee. Chronic haemodialysis is not recommended in the following groups of patients and prior consultation with a consultant nephrologist is required before offering haemodialysis treatment:-
 - i. Patients with terminal illnesses e.g terminal cancers, full blown AIDS, terminal end organ failure (liver, heart, lung etc).

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- ii. Patients with persistent hypotension.
 - iii. Patients with very poor quality of life e.g bed ridden, dementia, etc.
 - iv. Patients with extensive coronary disease and not suitable for any form of coronary intervention.
 - v. Patients with severe vascular disease where creation of any form of vascular access is unlikely to be successful. The patient and immediate family members shall be informed of the reasons for the decision and the patient may be offered alternative treatment if indicated.
5. Services are without discrimination without regards to race, colour, gender or national origin.
 6. Patients who undergo dialysis shall be charged according to the fees. (Medical Acts 1982).
 7. Patients shall be tested for biohazard status before initiating first haemodialysis treatment and after returning from another haemodialysis facility if necessary.

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APPENDIX 2

POLICY ON RENAL TRANSPLANTATION

1. Medical Criteria For Potential Living Related Renal Donors

- i. The donor must be between the age of 18 and 65 years.
- ii. There must be no history of diabetes, hypertension, malignancy, heart disease, renal disease, renal calculi and gout. There must be no history of drug addiction and high risk sexual behaviour.
- iii. Donor's body mass index of less than 30.
- iv. Donor must be sero negative for HIV and hepatitis B and C.
- v. The donors shall have acceptable results for the following laboratory investigations :
 - a. Appropriate ABO compatibility
 - b. Normal full blood count and coagulation profile
 - c. Normal urine full examination and microscopy
 - d. Normal renal profile
 - e. Creatinine clearance > 80 ml/min
 - f. 24 hour urine protein < 300mg
 - g. Normal liver function tests.
 - h. Anti-nuclear factor (ANF), anti-double-stranded deoxyribonucleic acid antibody (dsDNA) - negative.

2. Policy on Organ Transplantation

- i. All living donors shall be counselled by Nephrologist or doctor personal regarding the risk, benefit and possible consequences.
- ii. All staff to reinforce health education to donors and recipient.
- iii. All organ recipients require life-long immunosuppressive Drugs
- iv. All living organ donors shall be followed up for life.

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- v. Follow up care of patient who have undergone organ transplantation shall be provided by trained personal in SGH Kuching.
- vi. Donors workup shall be done in the respective unit. All blood investigation result and x-ray will be sent to SGH for review by the nephrologist.
- vii. All living donors who wish to donate their organs are to be referred to nephrologist for assessment.
- viii. All mentally disabled person shall be prohibited from live organ donation .

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APPENDIX 3

POLICY ON HAEMODIALYSIS TREATMENT AND OUTCOME

- i. Haemodialysis shall be performed three times weekly for at least 4 hours. Individual patients with specific problems may require more frequent dialysis.
- ii. Dialysis adequacy shall be estimated 3 monthly and the dialysis dose shall be adjusted to achieve a delivered *KTN* of at least 1.2 or a Urea Reduction Ratio of at least 65%.
- iii. Hepatitis seroconversion is a sentinel event and every case shall be notified and the infection control practice in the unit shall be thoroughly reviewed.
- iv. Death while on HD treatment or soon after shall be notified and an investigation be carried out to determine its cause.

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APPENDIX 4

HOSPITAL SERIAN CONSENT FORM FOR INITIATING DIALYSIS TREATMENT FOR NEW ESRD PATIENT

Patient's Name :
 Identity Card No. :
 Address :
 Age :
 Sex :
 Date :

I, the above named/parent/guardian/spouse/next of kin of above named, have been informed of the need for initiating dialysis of the patient. The attending renal trained staff has explained to me the risk, complication and benefits involved in the procedure as well as answering all my inquiries satisfactory. I understand that despite with close monitoring that have been done by the dialysis staff, there are still some unavoidable complications of dialysis that may be occur.

I fully understood the above and hereby agree to the dialysis treatment.

.....

Name and Signature of patient/
 Signature of attending
 Guardian / Spouse / next of kin

.....

Name and
 renal trained staff

I was present while the above matter was explained to the patient / guardian / spouse / next of kin whose signature appears above. In my opinion, the person referred to has understood the contents of this form and agree to have the dialysis treatment willingly.

.....

Signature of Witness
 Name of Witness:
 Identity Card No:

Adapted From :

1. Nephrology Services Operational Policy
2. HDU Operational Policy
3. The National Haemodialysis Quality and Standards 2018

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APPENDIX 5

INFECTION CONTROL IN THE HAEMODIALYSIS UNIT

- i. Proper hand washing technique.
- ii. Wear disposable gloves when caring for the patient or touching the patient's equipment at the dialysis station. Ensure a supply of clean non-sterile gloves and a glove discard container near each dialysis station.
- iii. Wash hand after gloves are removed and between patient contacts, as well as after touching blood, body fluids, secretions, excretions, and contaminated items.
- iv. A sufficient number of sinks with warm water and soap shall be available to facilitate hand washing.
- v. If hands are not visibly soiled, use of a waterless antiseptic hand rub can be substituted for hand washing.
- vi. Items taken to a patient's dialysis station, including those placed on top of dialysis machines, shall be disposed of, dedicated for use only single patient, or cleaned and disinfected before being returned to a common clean area or used for other patients.
- vii. Unused medications or supplies (e.g., syringes, alcohol swab) taken to the patient's station shall not be returned to a common clean area or used on their patients.
- viii. Prepare medications in room or area separated from the patient treatment area and designated only for medications.

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- ix. Do not handle or store contaminated (used supplies, used equipment, blood samples, or biohazard containers) in areas where medications and clean (unused) equipment and supplies are handled.
- x. Deliver medications separately to each patient. Common carts shall not to be used within the patient treatment area to prepare or distribute medications.
- xi. If trays are used to distribute medications, clean them before using for a different patient.
- xii. Intravenous medication vials labelled for single use, including erythropoietin, shall not be punctured more than once. Once a needle has entered a vial labelled for single use, the sterility of the product can no longer be guaranteed.
- xiii. Residual medication from two or more vials shall not be pooled into single vial.
- xiv. If a common supply cart is used to store clean supplies in the patient treatment area, this cart shall remain in a designated area at sufficient distance from patient stations to avoid contamination with blood. Such carts shall not be moved between stations to distribute supplies.
- xv. Staff members shall wear gowns, face shields, eye wear, or masks to protect themselves and prevent soiling of clothing when performing procedures during which sourting or spattering of blood might occur (e.g., during initiation and termination of dialysis, cleaning of dialyzers, and centrifugation of blood).
- xvi. Such protective clothing or gear shall be changed if it becomes soiled with blood,body fluids,secretions or excretions.

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- xvii. Staff members shall not eat,drink or smoke in the dialysis treatment area or in the laboratory.
- xviii. Patients can be served meals or eat food brought from home at their dialysis station.The glasses,dishes and other utensils shall be cleaned in the usual manner;no special care of these items is needed.

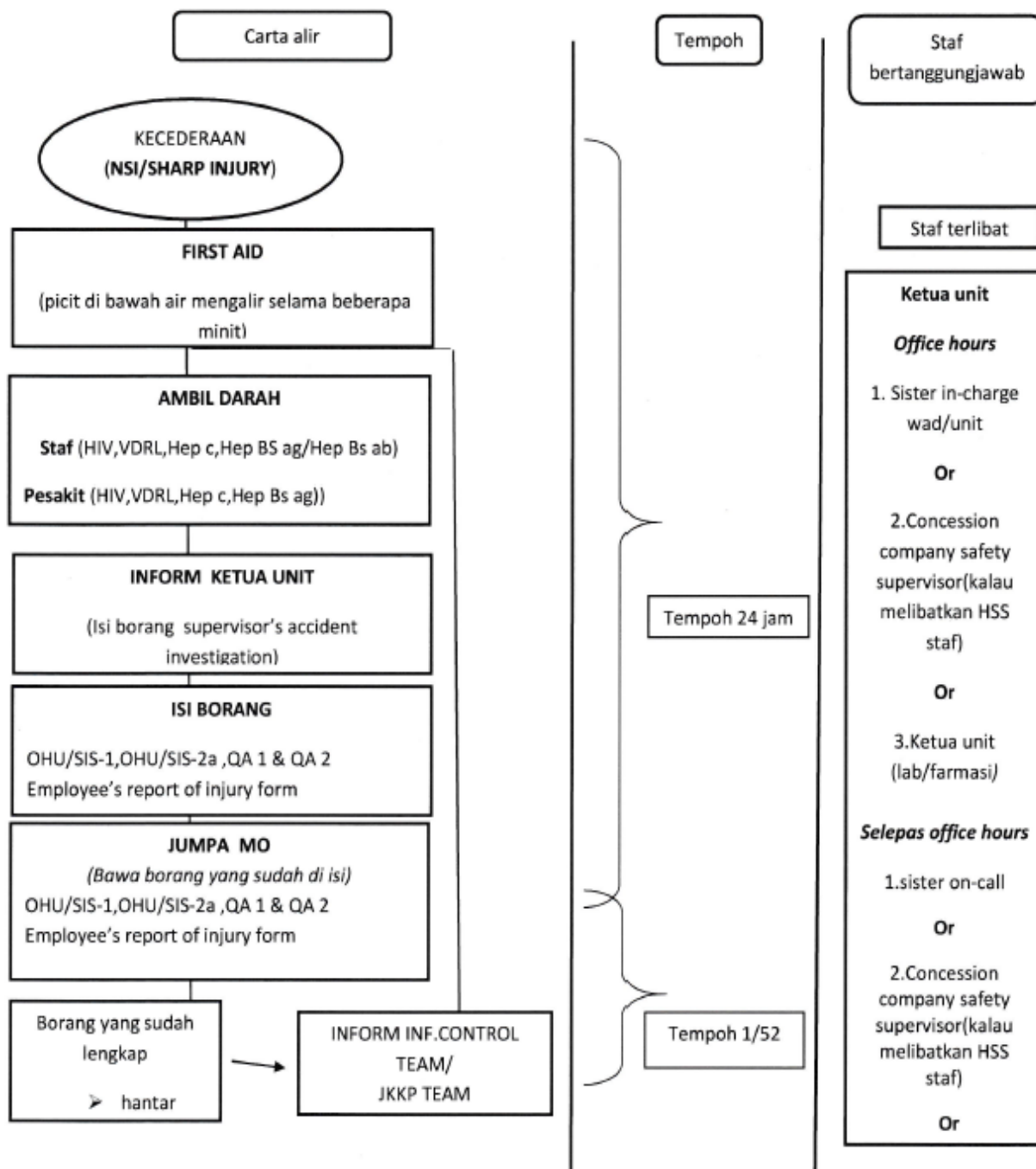
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APPENDIX 6

NEEDLE STICK INJURY PROTOCOL



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APPENDIX 7

POLICY ON SCREENING AND MONITORING BLOOD BORNE VIRAL INFECTIONS

1. All patients for haemodialysis shall be screened for viral infections: Prior to being accepted for treatment, patients shall be screened for HBsAg, anti HBs, anti HCV and anti HIV
2. HBsAg and anti-HCV shall be performed every 3 months
3. Anti-HIV shall be performed every 6 months
4. Patients who are HBsAg negative and do not have anti-HBs antibody shall be vaccinated. Patients with anti-HBs antibody titre less than 50 IU/L shall have a booster.

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APPENDIX 8

POLICY ON HANDLING OF INFECTED PATIENTS

POLICY FOR PATIENTS WITH HIV

1. The decision to provide long term haemodialysis to HIV patients shall be individualised.
2. These patients shall be dialysed separately and the dialyser shall NOT be reprocessed unless there is a dedicated HIV dialysis unit whereby dialysers of HIV patients can be reprocessed.
3. The disposal of blood lines and dialysers shall follow the MOH recommendations on disposal of clinical wastes.

POLICY FOR PATIENTS WITH HEPATITIS B POSITIVE

1. Patients with Hepatitis B shall be dialysed in a separate room with dedicated machines and reprocessing facilities.
2. Artificial kidney (dialyzer) to be used 12 times and to be reprocess by using the dialyser with Reprocessor provided.
3. Consumables items, cleaning equipment will be kept inside the room at all times.
4. Only authorised persons are allowed to enter the room.

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APPENDIX 9

POLICY AND PROCEDURES ON SCREENING OF THE CENTRE'S PATIENT WHO TEMPORARY DIALYSE OUTSIDE THE CENTRE

1. Patient should inform officer in charge before seeking treatment in other Hospital at least 2 week to 1 month before in advance.
2. Officer in charge to liase with receiving Hospital to find out available slot of the treatment session on the date that is to be fixed.
3. Referral letter and the latest blood test (at least 3 month) pass to patient to be given to the staff at the referral Hospital.
4. Patients need to bring their own micropore.

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APPENDIX 10

CATHETER RELATED BLOOD STREAM INFECTION REPORTING FORM

CENTRAL CATHETER RELATED INFECTION REPORTING AND SURVEILLANCE FORM HOSPITAL SERIAN

A. DEMOGRAPHIC DETAILS

Name:

IC:

MRN:

Date of admission:

Ward on admission:

Date of suspected infection:

B. CVC DETAILS

Date of insertion:

Type of CVC:

Insertion site:

Dates changed:

Previous history of CRBSI:

C. CULTURES SUMMARY

Date	Site	Isolated Organism	Sensitivity

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D. ANTIBIOTICS SUMMARY

Date	Antibiotics	Duration

Impression/Plan:

Prepared by,

Name:

Date:

ver 1.0

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APPENDIX 11

Maximum allowable levels of toxic chemical and dialysis fluid electrolytes in dialysis water.

Contaminant	Maximum Concentration (mg/l)	
Aluminium	0.01	
Total Chlorine	0.1	
Copper	0.1	
Fluoride	0.2	
Lead	0.005	
Nitrate (as N)	2	
Sulfate	100	
Zinc	0.1	
Electrolytes	Maximum Concentration	
	(mg/dl)	(mmol/l)
Calcium	2	0.05
Magnesium	4	0.15
Potassium	8	0.2
Sodium	70	3.0
Contaminants	Maximum Concentration (mg/l)	
Antimony	0.006	
Arsenic	0.005	
Barium	0.1	
Beryllium	0.0004	
Cadmium	0.001	
Chromium	0.014	
Mercury	0.0002	
Selenium	0.09	
Silver	0.005	
Thallium	0.002	

From ISO 23500:2011

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APPENDIX 12

LIST OF RECOMMENDED EQUIPMENT

Essential Medical and Non - Medical Equipment For Haemodialysis Unit

- i. Haemodialysis Machines
- ii. Dialysis Chair
- iii. Automated Dialyser Reprocessors
- iv. Water Purification Systems,Reverse Osmosis
- v. Automated Clotting Times (ACT)
- vi. Sitting weighing scale
- vii. Oxygen supply
- viii. Dressing trolleys
- ix. Emergency cart (trolley) with defibrillator)
- x. Non-invasive blood pressure monitoring (NIBP) sets
- xi. Pulse oxymeters
- xii. Volumetric infusion pumps.
- xiii. Electrocardiographs (ECG) machine
- xiv. Glucose monitoring set
- xv. Refrigerator
- xvi. Computer with internate connectivity

Optional items for Haemodialysis Unit

- i. Online Haemodiafiltration machine (HDF)
- ii. Continious Renal replacement Therapy (CRRT) Machine
- iii. Portable reverse Osmosis Water System
- iv. Non Invasive Vascular Access Monitoring
- v. Slow Extended daily Dialysis Machine (SLEDD)
- vi. Dialyser rinsing machines
- vii. Bioimpedance analyser

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APPENDIX 13

QUALITY IMPROVEMENT ACTIVITIES

- i. Daily 'self test' for hemodialysis machine
- ii. After each hemodialysis machine usage, hot disinfection must be carried out at of the day.
- iii. Germicide daily test done on reserve osmosis water and to be recorded.
- iv. To replace guard filter of RO water system each month or if PSI below 15
- v. Refill 10 kg salt twice a week into brine tank for regeneration of column softener to ensure water conductivity.
- vi. Water samples to be tested every 6 months to ensure RO water quality comply AAMI standards.
- vii. Monthly test for chloramines, chlorine, water ph, "microbiology" and "culture and sensitivity".
- viii. Daily calibration done every morning for dialyser reprocessing system **(RDS)** to ensure its function and safety.
- ix. Sanitation to dialyser reprocessor system (RDS) to be done end of the day.
- x. Checking of renalin level in dialyser to ensure good disinfection and reprocessing.
- xi. Residual renalin must be checked after priming blood lines to ensure free from renalin. This step is crucial before each hemodialysis to ensure patient safety.
- xii. Biozard test to done 6 monthly for all patients
- xiii. HDU staff must be screened yearly for hepatitis B infections
- xiv. Patients satisfaction survey to be done twice a year
- xv. Refrigerator temperature check daily and to be recorded.

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APPENDIX 14

KEY ASPECTS OF WHOLE HOSPITAL POLICIES

• Domestic Service

- i. General cleaning and cleansing of the unit shall be carried out by **HSS**.

• Movement of Patients

- i. Movement of in-patient is done by ward staff, while movement of out patient is done by HDU Nurse/**AMO/PPK**.

• Movement of Supplies

- i. Medical / Surgical / Pharmaceutical shall be indented from the Pharmacy by HDU. These supplies shall be checked at the issuing areas in the store by the designed officer before being brought back to the unit.
- ii. All Medical supplies will be kept and in the unit Store.

• Waste Management

- i. Clinical and domestic waste shall be bagged in separate colour codes bags. **HSS** shall collect according to schedule.

• Laundry Service

- i. Soiled and clean linen shall be done by **HSS**.

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APPENDIX 15

CODE BLUE PROTOCOL

CODE BLUE RESPONSE

1. A Rapid response system for emergency resuscitation and stabilization of any medical Emergency that happen within the hospital area and outside the clinical areas.

2. Objective

- 2.1 Provide rapid, almost immediate resuscitation and stabilization for victim of any medical emergencies or cardio respiratory arrest within the hospital ground.
- 2.2 To initiate training in BLS skills and use of automated External defibrillator (AED) for all hospital staff whether clinical or non-clinical based.

3. When To Initiate.

A code blue need to be initiated immediately whene ver a person is found in cardiac or respiratory arrest (unresponsive. pulseless or not breathing) that require cardio pulmonary resuscitation (CPR).

4. Who Involved.

Specialized and well-trained team from ETD.

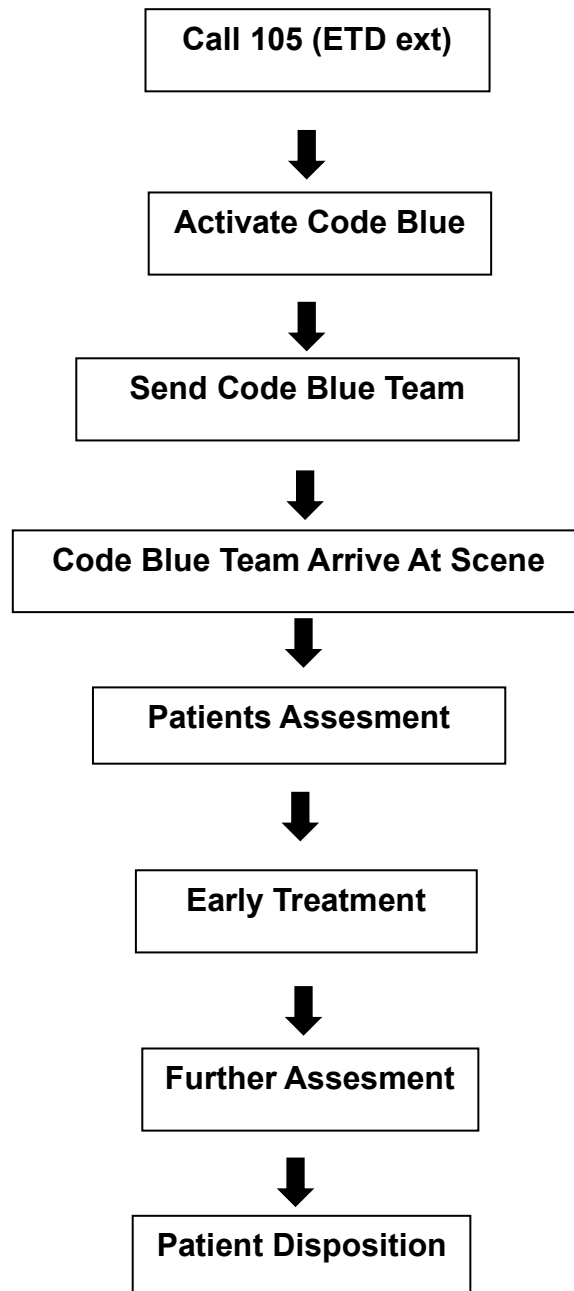
5. What To Do.

- 5.1 Activate code blue system contact ETD.
- 5.2 Anival of the code blue team (in charge of the area to Perform BLS).
- 5.3 Decision making on victim disposition.

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6. Code Blue Flow Chart.



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APPENDIX 16

Policy on Minimum Laboratory Tests done for Haemodialysis Patients

Tests	Frequency
Full Blood Count	Every 3 monthly
Iron Study: Serum Iron Serum Ferritin Total Iron Binding Capacity(TIBC) Iron saturation (Tsats)	Every 3 monthly
Blood Urea (Pre & Post dialysis)	Every 3 monthly
Renal Function test	Every 3 monthly
Liver Function test Alanine Transaminases Alkaline Phosphate Serum Albumin	Every 3 monthly Consider monthly transaminases for 3 month in patients who have been dialyzing elsewhere or patients who received blood transfusion.
Calcium & Phosphate	Every 3 monthly
Fasting IPTH	Every 3-6 monthly
Fasting Serum Lipid	Every 6 monthly
Blood Sugar	Every 3 monthly (diabetic) Every 3-6 monthly (non-diabetic)
HbA1C (if diabetic)	Every 3-6 monthly
Virology Hep B s Ag Anti Hep B s antibody titer Anti HCV Anti HIV	Every 3 monthly(annually if Hep B s Ag is positive) Every 6 monthly Every 3 monthly (if anti HCV negative) Every 3 monthly

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2. HDU Operational Policy
3. Haemodialysis Quality And Standards.

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